

pharmaniaga®

Paracetamol

 Tablet

DESCRIPTION

White, oblong-shaped tablets with Pharmaniaga icon on one side and breakline on the other.

COMPOSITION

Each tablet contains Paracetamol 650 mg and Potassium sorbate 0.75 mg as preservative.

PHARMACODYNAMICS

Pharmacotherapeutic group: Other analgesics and antipyretics, Anilides

ATC code N02B E01

Paracetamol is an antipyretic analgesic. The mechanism of action is probably similar to that of aspirin and dependant on the inhibition of prostaglandin synthesis. This inhibition appears, however to be on a selective basis.

PHARMACOKINETICS

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract. The concentration in plasma reaches a peak in 30 to 60 minutes and the plasma half-life is 1 - 4 hours after therapeutic doses. Paracetamol is relatively uniformly distributed throughout most body fluids. Binding of the drug to plasma proteins is variable; 20 to 30% may be bound at the

concentrations encountered during acute intoxication. Following therapeutic doses 90 - 100% of the drug may be recovered in the urine within the first day. However, practically no paracetamol is excreted unchanged and the bulk is excreted after hepatic conjugation.

INDICATIONS

Paracetamol is used for the symptomatic relief of fever, minor aches, pains and backache.

CONTRAINDICATIONS

Hypersensitivity to paracetamol or any of the other ingredients/components of the product.

Severe and active hepatic impairment.

ADVERSE REACTIONS

Adverse reactions of paracetamol are rare and usually mild, though hematological reactions have been reported.

Body system	Adverse reactions	Frequency
Blood and lymphatic system disorders	Thrombocytopenia	Very rare
	Agranulocytosis	
Immune system disorders	Anaphylaxis	Very rare
	Cutaneous hypersensitivity reactions including skin rashes and angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.	Unknown
Respiratory, thoracic and mediastinal disorders	Bronchospasm*	Very rare
Hepatobiliary disorders	Hepatic dysfunction	Very rare

*There have been cases of bronchospasm with paracetamol, but these are more likely in asthmatics sensitive to aspirin or other NSAIDs.

WARNING AND PRECAUTION

This preparation contains PARACETAMOL. Do not take any other PARACETAMOL containing medicines at the same time.

Allergy alert

Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of serious condition. If these reactions occur, stop use and seek medical assistance right away.

- Keep out of reach of children.
- Do not take if allergic to paracetamol.
- Patients should contact their health care provider if symptoms persist (if the pain lasts for more than 10 days, if there is redness or fever lasts more than 3 days).
- Paracetamol should be given with care to patients with impaired kidney or liver function.
- Large doses should be avoided in patients with hepatic impairment. Paracetamol overdose may harm the liver.
- Do not exceed recommended dose.
- It should be given with care to patients with alcohol dependence.
- Paracetamol provides symptomatically, with relief only, additional therapy to treat the cause pain or fever should be initiated when necessary.
- Patients should be advised to consult their doctor if their headaches become persistent.
- Patients should be advised to consult a doctor if they suffer from non-serious arthritis and need to take painkillers every day.
- Caution should be exercised in patients with glutathione depleted states, as the use of paracetamol may increase the risk of metabolic acidosis.
- Use with caution in patients with glutathione depletion due to metabolic deficiencies.

PREGNANCY AND LACTATION

Pregnancy

Considered to be the analgesic of choice in pregnant patients.

Although it crosses placenta, paracetamol should be used at the lowest effective dose for the shortest possible time as a minor analgesic/antipyretic in pregnancy.

Lactation

Excreted in breast milk.

Maternal ingestion of paracetamol in normal therapeutic doses does not appear to present a risk to the nursing infant.

Available published data do not contraindicate breastfeeding.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

It is unlikely to impair a patient's ability to drive or use machinery.

INTERACTION WITH OTHER MEDICAMENTS

The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by colestyramine.

RECOMMENDED DOSAGE

Adults: 1 to 1 ½ tablets three or four times a day up to a maximum of 6 tablets daily.

Children should not be given paracetamol for more than 3 days without consulting a doctor.

These doses should not be repeated more frequently than every four hours nor should more than four doses be given in any 24 hour period.

ROUTE OF ADMINISTRATION

Oral.

OVERDOSAGE

Liver damage is possible in adults who taken 10g or more of paracetamol. Ingestion of 5g or more of paracetamol may lead to liver damage if the patient has risk factors (see below).

Paracetamol overdose may cause liver failure which may require liver transplant or lead to death.

Risk factors:

If the patient:-

- is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes.

Or

- regularly consumes ethanol in excess of recommended amounts.

Or

- is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms:

Symptoms of paracetamol overdose in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Treatment:

Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patient should be referred to hospital urgently

for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section.

Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous-N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24h from ingestion should be discussed with the NPIS or a liver unit.

STORAGE CONDITIONS

Store below 30°C. Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In blisters of 30, 100 and 500 tablets.

Date of revision: 13 June 2022

PRODUCT REGISTRATION HOLDER/**MANUFACTURER:****PHARMANIAGA MANUFACTURING****BERHAD (198001006232)**

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